Patient ID	101301186	Patient Name	[redacted]
Age	38 Years	Sex	F
Accession No.		Modality	US
Ref. Physician		Study	
Study Date	11-Feb-2026		

ULTRASOUND WHOLE ABDOMEN

Sub-optimal evaluation due to excessive bowel gas shadows.

Liver is normal in size (13.4cm) **& shows raised periportal echogenicity - ? Subjective ?? Viral etiologuy**, IHBR are not dilated. PV & CBD are of normal caliber. No focal lesion is seen.

Gall bladder *is partially distended. Visualized lumen appear echo free.*

Pancreas head and proximal body is normal, rest is obscured by bowel gas.

Spleen is normal in size (10.4cm) & has homogenous parenchyma.

Bilateral kidneys are normal in size, shape & position. Corticomedullary differentiation is maintained. Pelvicalyceal system are not dilated. No echogenic shadow suggestive of calculus is seen.

Urinary bladder *is partially distended. Pelvis could not be assessed.*

No significant retroperitoneal lymphadenopathy is seen.

No free fluid is seen in the peritoneal cavity.

IMPRESSION:

- **No significant abnormality seen at present evaluation.**

Please correlate clinically.

[redacted]

(Junior Resident II)

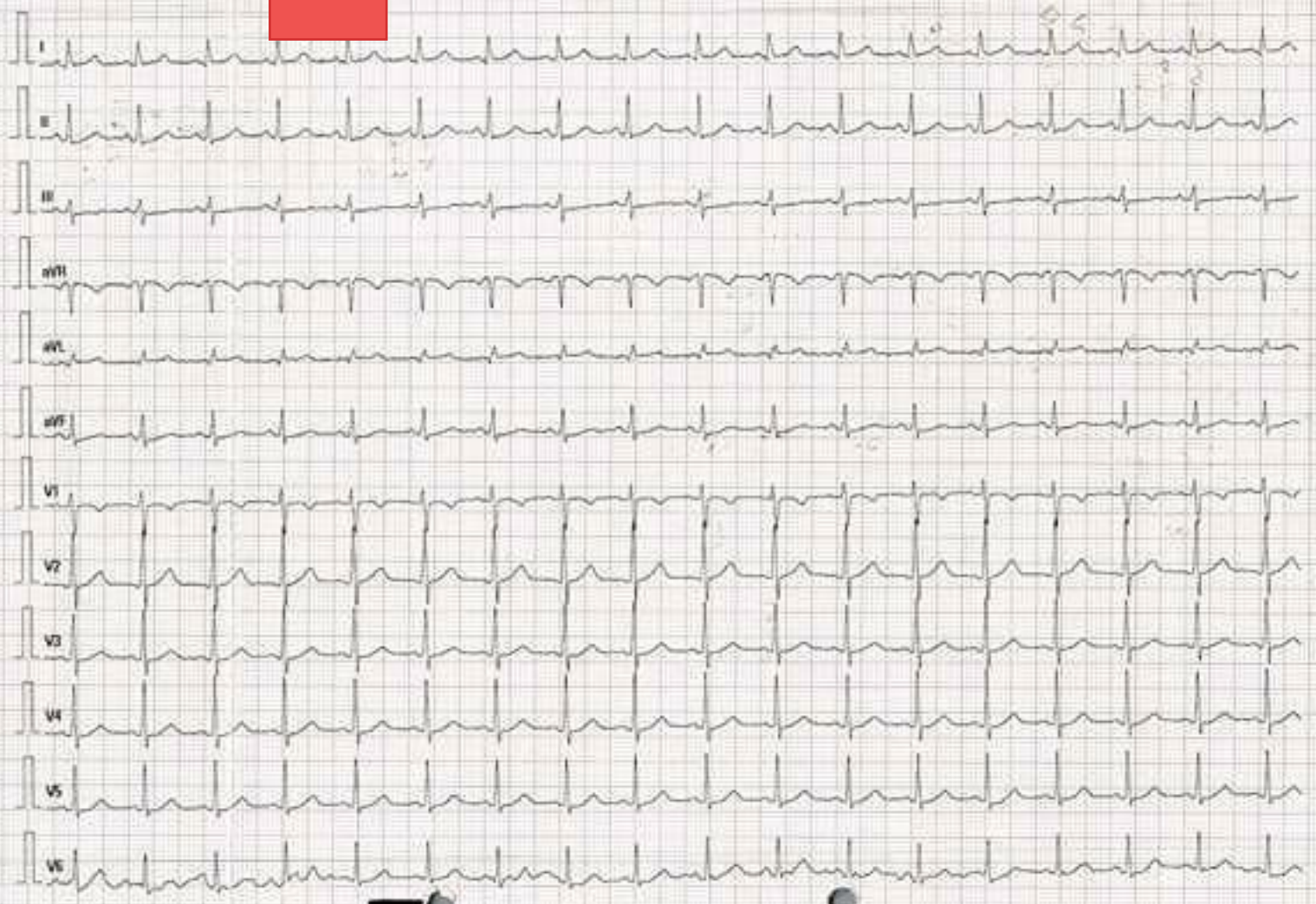
[redacted]

11-Feb-2026 03:46:10 PM

[redacted]

ID: 20260211161451990 11-02-2026 05:55:29 PM Vent. Rate (bpm) 107

Name: [redacted]



25 mm/s 10 mm/mV 50 Hz [redacted] [redacted] 02.07.00/V28.4.1 SN:FN-60013327

ID: 20260211161 46/990
Name:
Age:
Gender:

[redacted]

11-02-2026 05:55:39 PM

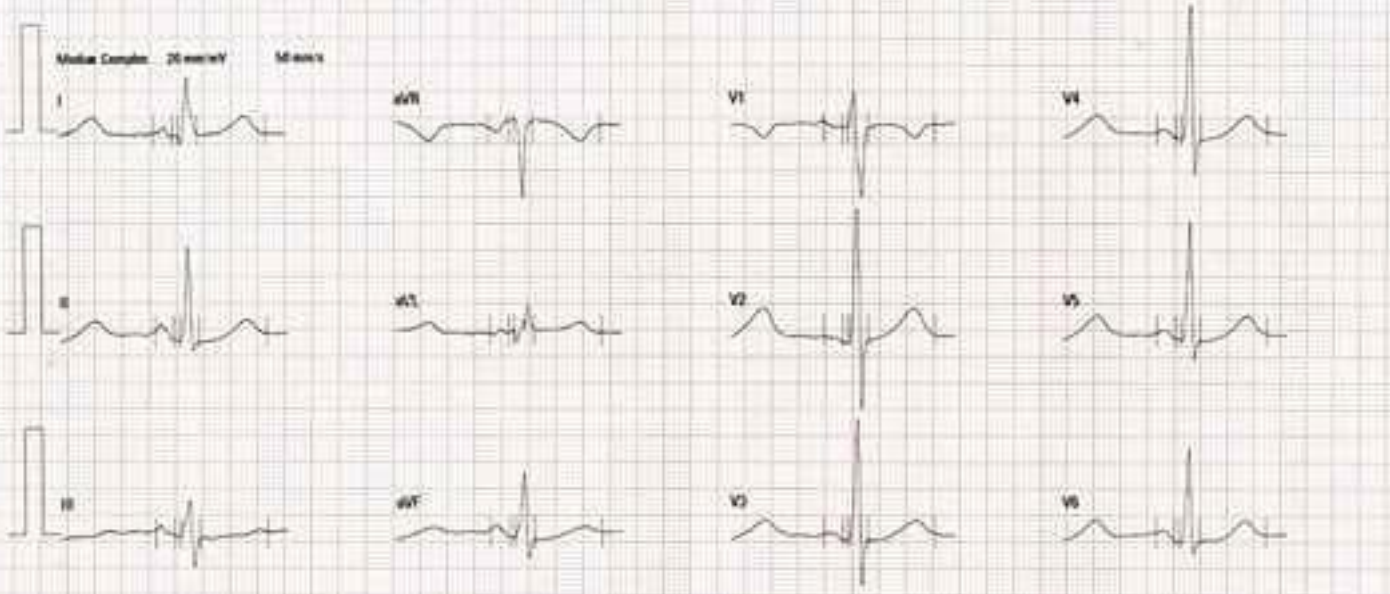
Vent. Rate	107 bpm
PR Interval	90 ms
QRS Duration	72 ms
QT/QTc Interval	322/404 ms
P/QRS/T Axes	64/41/41 deg

QTc Hodges

Sinus tachycardia
--- Interpretation made without knowing patient's gender/age ---
Short PR interval

Borderline ECG

Unconfirmed Diagnosis



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& RESEARCH INSTITUTE

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CENTRAL LABORATORY

Patient Name	[redacted]	**Lab No**	1411957
Reg No/IP No	101301186 / 26/110098	**Sample Date**	12/02/2026 22:27
Age/Gender	38 Yrs/Female	**Receiving Date**	13/02/2026 01:05
Bed No/Ward	General Medicine	**Report Date**	13/02/2026 09:26
Referred By	Dr. General Medicine Unit III	**Report Status**	Final

[redacted]

Test Name	Result	Unit	Biological Ref. Range	Method
CBC (COMPLETE HAEMOGRAM)				
Sample: Whole Blood EDTA				
Haemoglobin (Hb)	**6.3 L**	gm/dl	12.0 - 15.0	Colorimetric
Total Leukocyte Count (TLC)	**3350 L**	cells/cumm	4000 - 10000	SF Cube Cell method
DLC				
Neutrophils	55	%	40.0 - 80.0	Flow Cytometry
Lymphocytes	36	%	20.0 - 40.0	Flow Cytometry
Eosinophils	04	%	1.0 - 6.0	Flow Cytometry
Monocytes	05	%	2.0 - 10.0	Flow Cytometry
Basophils	00	%	0.0 - 0.5	Flow Cytometry
Platelet Count	1.58	lakh/cumm	1.50 - 4.10	Electrical Impedance Method
RBC Count (Red Blood Cell)	**1.57 L**	10^6/uL	3.80 - 4.80	Electrical Impedance Method
Hematocrit - (Hct.)	**18.4 L**	%	36 - 46	CALCULATED
MCV	**117.3 H**	fl	83.0 - 101.0	CALCULATED
MCH	**40.1 H**	pg	27.0 - 32.0	CALCULATED
MCHC	34.2	gm/dl	31.5 - 34.5	CALCULATED
RDW	**33.2 H**	%	11.6 - 14.0	Electrical Impedance Method

CBC:
A complete blood count is a blood panel that gives information about the cells in a patient's blood, such as the cell count for each cell type and the concentrations of Hemoglobin and platelets. The cells that circulate in the bloodstream are generally divided into three types: white blood cells (leukocytes), red blood cells (erythrocytes), and platelets (thrombocytes). Abnormally high or low counts may be physiological or may indicate disease conditions, and hence need to be interpreted clinically.

--End Of Report--

[redacted]

Printed [redacted]

[redacted]

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CENTRAL LABORATORY

Patient Name	[redacted]	**Lab No**	1409349
Reg No/IP No	101301186 / 26/110098	**Sample Date**	12/02/2026 09:54
Age/Gender	38 Yrs/Female	**Receiving Date**	12/02/2026 12:00
Bed No/Ward	General Medicine	**Report Date**	12/02/2026 14:58
Referred By	Dr. General Medicine Unit III	**Report Status**	Final

[redacted]

Test Name	Result	Unit	Biological Ref. Range	Method
FOLIC ACID				
Sample: Serum				
Folic Acid	3.18	ng/ml	3 - 20	Chemiluminescent Immuno Assay
Normal			3.1 - 17.5 ng/mL	
Borderline deficient			2.2 - 3.0 ng/mL	
Deficient			< 2.2 ng/mL	
Excessive			> 17.5 ng/mL	
VITAMIN B12				
Sample: Serum				
Vitamin B12	**159 L**	pg/ml	239.00 - 931.00	Chemiluminescent Immunoassay (CLIA)

[redacted]

[redacted]

Patient Name		**Lab No**	1409349
Reg No/IP No	101301186 / 26/110098	**Sample Date**	12/02/2026 09:54
Age/Gender	38 Yrs/Female	**Receiving Date**	
Bed No/Ward	General Medicine	**Report Date**	
Referred By	Dr. General Medicine Unit III	**Report Status**	Final

Newborn 160-1300

Adult 200-835

Adult >60Y 110-800

Comment :

Vitamin B 12 (Cobalamin) is an important water -soluble vitamin. In contrast to other water -soluble vitamins it is not excreted quickly in the urine, but rather accumulates and is store din the liver, kidney and other body tissues. Humans obtain vitamin B12exclusively from anim dietary sources, such as meat, eggs and milk. As a result, a vitamin B 12 deficiency may not manifest itself until after 5 or 6 years of a die supplying in a dequate amounts. Vitamins B12 functions as a methyl donor and work with folic acid in the synthesis of DNA and red blood and is vitally important in maintaining the health of the insulation sheath (myelin sheath) that surrounds nerve cells. Preservatives such a fluorides & ascorbic acid interfere with this assay. Excessive exposure of the specimen to light may alter Vitamin B12 result.

Kindly correlate with clinical conditions.

--End Of Report--

Patient Name		**Lab No**	1409222
Reg No/IP No	101301186 / 26/110098	**Sample Date**	11/02/2026 22:21
Age/Gender	38 Yrs/Female	**Receiving Date**	12/02/2026 04:04
Bed No/Ward	General Medicine	**Report Date**	12/02/2026 09:38
Referred By	Dr. General Medicine Unit III	**Report Status**	Final

Interpretation:-
This is a Screening test. A Negative result implies that no anti HIV 1 or anti HIV 2 antibodies have been detected in a sample by this method. However, it does not rule out possibility of exposure to or infection with HIV 1 or 2. This could also mean that either patient has not been exposed to HIV 1 and 2 or sample has been tested during window phase(Before development of detectable antibodies). Therefore, for a subject suspected of HIV Exposure, it is advisable to repeat the test after 3 months.

Note:- The report has been released after testing as per NACO strategies.

ANTI HEPATITIS C (HCV) CARD

Sample: Serum

Antibody to Hepatitis C Virus	NON-REACTIVE	Immunochromatography

Interpretation:
1. This is a screening test. A non-reactive result does not exclude the possibility of exposure or infection with HCV.
2. The presence of anti HCV may indicate recent infection or past infection by HCV. HCV antibodies may be undetectable in some stage of infection & in some clinical conditions.
3. False positive results are seen in Autoimmune diseases, Rheumatoid factor, Hypergamma-globulinemia, Para proteinemia, passive antibody transfer, Anti-idiotypes & Anti-superoxide dismutase.
4. False negative results are seen in early Acute infection, Immuno-suppression & Immuno-incompetence.
* All reactive samples should be confirmed by confirmatory test like ECI/ RIBA.

--End Of Report--

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CENTRAL LABORATORY

Patient Name		**Lab No**	1409230
Reg No/IP No	101301186 / 26/110098	**Sample Date**	11/02/2026 23:01
Age/Gender	38 Yrs/Female	**Receiving Date**	12/02/2026 06:21
Bed No/Ward	General Medicine	**Report Date**	12/02/2026 09:55
Referred By	Dr. General Medicine Unit III	**Report Status**	Final

Test Name	Result	Unit	Biological Ref. Range	Method
LDH				
Sample: Serum				
LDH	**2536 H**	U/L	120 - 246	L to P - IFCC

--End Of Report--

CENTRAL LABORATORY

Patient Name		**Lab No**	1409230
Reg No/IP No	101301186 / 26/110098	**Sample Date**	11/02/2026 23:01
Age/Gender	38 Yrs/Female	**Receiving Date**	12/02/2026 05:42
Bed No/Ward	General Medicine	**Report Date**	12/02/2026 07:01
Referred By	Dr. General Medicine Unit III	**Report Status**	Final

Test Name	Result	Unit	Biological Ref. Range	Method
Blood Group (ABO and Rh)	"O" RH Positive			Gel Card Method
Comment	In case of any discrepancy, Kindly repeat the test in Blood Bank.			

In case of any discrepancy, Kindly repeat the test in Blood Bank.

ESR

Sample: Whole Blood EDTA

ESR (Erythrocyte Sedimentation Rate)	5	mm/1st hr	< 20	Opto Elecronic Method

ESR measures the rate at which the RBCs settle at the bottom of a test tube. A higher than normal ESR indicates an underlying inflammatory process which can be seen in infectious conditions like tuberculosis etc, anemia, multiple myeloma, rheumatoid arthritis, rheumatic fever, vasculitis, kidney diseases, various malignancies and physiological conditions like pregnancy. Lower than normal ESR can be due to polycythaemia, sickle cell anemia, spherocytosis etc.

--End Of Report--

CENTRAL LABORATORY

Patient Name		**Lab No**	1409230
Reg No/IP No	101301186 / 26/110098	**Sample Date**	11/02/2026 23:01
Age/Gender	38 Yrs/Female	**Receiving Date**	12/02/2026 02:36
Bed No/Ward	General Medicine	**Report Date**	12/02/2026 02:49
Referred By	Dr. General Medicine Unit III	**Report Status**	Final

Test Name	Result	Unit	Biological Ref. Range	Method
COOMB'S TEST DIRECT				
Sample: Whole Blood EDTA				
Direct Coombs Test (DCT)	Negative		Negative	Manual (ID Card)
COOMB'S TEST INDIRECT				
Sample: Serum				
Indirect Coombs Test (ICT)	Negative		Negative	Manual (ID Card)

--End Of Report--

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CENTRAL LABORATORY

Patient Name		**Lab No**	1409269
Reg No/IP No	101301186 / 26/110098	**Sample Date**	12/02/2026 00:56
Age/Gender	38 Yrs/Female	**Receiving Date**	12/02/2026 12:22
Bed No/Ward	General Medicine	**Report Date**	12/02/2026 14:57
Referred By	Dr. General Medicine Unit III	**Report Status**	Final

Test Name	Result	Unit	Biological Ref. Range	Method
Stool Occult Blood	Negative		Negative	Guaiac (Card test)

--End Of Report--

Patient Name		**Lab No**	1409222
Reg No/IP No	101301186 / 26/110098	**Sample Date**	11/02/2026 22:21
Age/Gender	38 Yrs/Female	**Receiving Date**	12/02/2026 04:04
Bed No/Ward	General Medicine	**Report Date**	12/02/2026 09:38
Referred By	Dr. General Medicine Unit III	**Report Status**	Final

Test Name	Result	Unit	Biological Ref. Range	Method
HEPATITIS B SURFACE ANTIGEN (HBSAG) BY CARD				
Sample: Serum				
Hepatitis B surface Antigen	NON-REACTIVE			Immunochromatography

INTERPRETATION :

NON REACTIVE: This result implies is NON REACTIVE for HBsAg
PROVISIONALLY REACTIVE / BORDERLINE REACTIVE: This result Suggests the possibility of HBsAg infection.
However these results must be verified by confirmatory tests.
Note: Limitations of the procedure :
1.This Test should be used for the detection of HBsAg in serum or plasma only and not in other fluids
2.This is only screening test All reactive samples should be confirmed by confirmatory tests. Therefore for a definitive diagnosis, the paitent's clinical history, symptomogy as well as serological data be considered. The results should be reported only after complying with above procedure.
3.Additional Follow-up Testing using available clinical methods (along with test) is required if this screening test is non reactive with persisting clinical symptoms.
4.False positive results can be obtained due to the presence of the antigens or elevated levels of RF factor. This occur in less than 1% of the samples tested.

HIV I & II (BY CARD) P				
Sample: Serum				
Test 1 :- HIV TRIDOT	Non-Reactive HIV-1 & HIV 2 antibodies			Membrane Immuno-Filteration
INTERPRETATION OF RESULTS	Specimen is Negative for HIV antibodies			

CENTRAL LABORATORY

Patient Name		**Lab No**	1409222
Reg No/IP No	101301186 / 26/110098	**Sample Date**	11/02/2026 22:21
Age/Gender	38 Yrs/Female	**Receiving Date**	12/02/2026 05:44
Bed No/Ward	General Medicine	**Report Date**	12/02/2026 10:21
Referred By	Dr. General Medicine Unit III	**Report Status**	Final

Test Name	Result	Unit	Biological Ref. Range	Method
CBC (COMPLETE HAEMOGRAM)				
Sample: Whole Blood EDTA				
Haemoglobin (Hb)	**5.5 L**	gm/dl	12.0 - 15.0	Colorimetric
Total Leukocyte Count (TLC)	**3510 L**	cells/cumm	4000 - 10000	SF Cube Cell method
DLC				
Neutrophils	52	%	40.0 - 80.0	Flow Cytometry
Lymphocytes	**42 H**	%	20.0 - 40.0	Flow Cytometry
Eosinophils	03	%	1.0 - 6.0	Flow Cytometry
Monocytes	03	%	2.0 - 10.0	Flow Cytometry
Basophils	00	%	0.0 - 0.5	Flow Cytometry
Platelet Count	1.75	lakh/cumm	1.50 - 4.10	Electrical Impedance Method
RBC Count (Red Blood Cell)	**1.20 L**	10^6/uL	3.80 - 4.80	Electrical Impedance Method
Hematocrit - (Hct.)	**16.8 L**	%	36 - 46	CALCULATED
MCV	**140.2 H**	fl	83.0 - 101.0	CALCULATED
MCH	**46.0 H**	pg	27.0 - 32.0	CALCULATED
MCHC	32.7	gm/dl	31.5 - 34.5	CALCULATED
RDW	**15.0 H**	%	11.6 - 14.0	Electrical Impedance Method

CBC:
A complete blood count is a blood panel that gives information about the cells in a patient's blood, such as the cell count for each cell type and the concentrations of Hemoglobin and platelets. The cells that circulate in the bloodstream are generally divided into three types: white blood cells (leukocytes), red blood cells (erythrocytes), and platelets (thrombocytes). Abnormally high or low counts may be physiological or may indicate disease conditions, and hence need to be interpreted clinically.

--End Of Report--

CENTRAL LABORATORY

Patient Name		**Lab No**	1409222
Reg No/IP No	101301186 / 26/110098	**Sample Date**	11/02/2026 22:21
Age/Gender	38 Yrs/Female	**Receiving Date**	12/02/2026 05:43
Bed No/Ward	General Medicine	**Report Date**	12/02/2026 10:21
Referred By	Dr. General Medicine Unit III	**Report Status**	Final

Test Name	Result	Unit	Biological Ref. Range	Method
PERIPHERAL SMEAR REPORT				
Sample: Whole Blood EDTA				
PERIPHERAL SMEAR EXAMINATION				
WBCs	TLC-3,510cells/cumm DLC- N52 L42 E03 M03 B00 Hypersegmented neutrophils seen.			
PLATELETS	1,75,000/microliter Counts are adequate in number and normal in morphology.			
RBCs	Reduced red cell mass. Predominantly macrocytic RBCs are seen. Occasional tear drop cells seen.			
Peripheral Smear Examination for Micro Filaria	No hemoparasite seen in the smear studied.			Leishman Staining and Light Microscopy
IMPRESSION	Macrocytic anemia.			
Advice :	Serum vitB12 and serum folate studies.			
RETICULOCYTE COUNT				
Sample: Whole Blood EDTA				
Reticulocyte Count	2.0	%	0.5 . 2.5	Light Microscopy using Supravital Dye

Comments:
1) The number of Reticulocytes is a good indicator of Bone Marrow activity. It represents recent production and allows for the determination of the Reticulocyte Production Index (RPI). These values can be used to determine whether a production problem is contributing to the Anemia and can also be used to monitor the progress of treatment for Anemia.
2) A very high number of Reticulocytes in the blood can be described as Reticulocytosis, as seen in cases of Haemolytic Anaemia.
3) Abnormally low numbers of Reticulocytes can be attributed to chemotherapy, Aplastic Anemia, Pernicious Anemia, Bone Marrow malignancies, problems of Erythropoietin production, various Vitamin or mineral deficiencies (Iron, Vitamin B12, Folic Acid) disease states (Anemia of chronic disease) and other causes of Anemia due to poor RBC production.

--End Of Report--

[redacted]

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CENTRAL LABORATORY

Patient Name	[redacted]	**Lab No**	1409222
Reg No/IP No	101301186 / 26/110098	**Sample Date**	11/02/2026 22:21
Age/Gender	38 Yrs/Female	**Receiving Date**	12/02/2026 06:21
Bed No/Ward	General Medicine	**Report Date**	12/02/2026 11:01
Referred By	Dr. General Medicine Unit III	**Report Status**	Final

[redacted]

Test Name	Result	Unit	Biological Ref. Range	Method
KIDNEY FUNCTION TEST (KFT)				
Sample: Serum				
BUN	**6.7 L**	mg/dl	7 - 17	Urease
Blood Urea	14.47	mg/dl		CALCULATED
Creatinine	**0.49 L**	mg/dl	0.52 - 1.04	Enzymatic
Uric Acid	3.9	mg/dl	2.5 - 6.2	Uricase
Protein Total	7.60	g/dl	6.30 - 8.20	Biuret Method
Calcium	8.9	mg/dl	8.4 - 10.2	Arsenazo
Phosphorus	3.3	mg/dl	2.5 - 4.5	Phosphomolybdate
Sodium	141	mmol/L	137 - 145	ISE Direct
Potassium	4.5	mmol/L	3.5 - 5.1	ISE Direct
Chloride	105	mEq/L	98 - 107	ISE Direct

--End Of Report--

[redacted]

[redacted]

CENTRAL LABORATORY

Patient Name		**Lab No**	1409222
Reg No/IP No	101301186 / 26/110098	**Sample Date**	11/02/2026 22:21
Age/Gender	38 Yrs/Female	**Receiving Date**	12/02/2026 07:08
Bed No/Ward	General Medicine	**Report Date**	12/02/2026 08:05
Referred By	Dr. General Medicine Unit III	**Report Status**	Final

Test Name	Result	Unit	Biological Ref. Range	Method
URINE R/M				
Sample: Urine				
PHYSICAL EXAMINATION				
Volume	30	ml	10 - 30	
Color	Pale Yellow		Pale Yellow	
Appearance	Clear		Clear	
CHEMICAL EXAMINATION (By Dipstick method)				
pH	6.0		5.0 - 7.5	Double Indicator Principle
Specific Gravity	1.020		1.010 - 1.030	Ion Exchange
URINE ALBUMIN	NIL			
Urine Sugar	NIL			
Ketone	Negative			Sodium Nitroprusside Reaction
Urobilinogen	NIL			
Bilirubin	NIL			
Nitrite	NIL			
Blood	NIL			
Leucocytes (Esterase)	NIL			
MICROSCOPIC EXAMINATION				
RBC	NIL	/hpf	Nil	Light Microscopy on Centrifuged Sediment
Leucocyte	2-3		0-5	
Epithelial Cells	3-4	/hpf	0-5	
Crystals	Absent		Absent	
Casts	Absent		Absent	
Bacteria	Absent		Absent	

--End Of Report--

Patient Name		**Lab No**	1409222
Reg No/IP No	101301186 / 26/110098	**Sample Date**	11/02/2026 22:21
Age/Gender	38 Yrs/Female	**Receiving Date**	12/02/2026 06:21
Bed No/Ward	General Medicine	**Report Date**	12/02/2026 11:01
Referred By	Dr. General Medicine Unit III	**Report Status**	Final

Test Name	Result	Unit	Biological Ref. Range	Method
FERRITIN				
Sample: Serum				
Ferritin	**202.0 H**	ng/ml	6.2 - 137.0	Chemiluminescent Immuno Assay

Interpretation-

The Ferritin blood test measuresthe level of ferritin in the blood. Ferritin is a protein inside your cellsthat stores iron. It allows your body to use the iron when it needs it. Aferritin test indirectly measures the amount of iron in your blood. Ferritin isa compound composed of iron molecules bound to Apoferritin, a protein shell,Stored iron represents about 25% of total iron in the body , and most of thisiron is stored as Ferritin, Ferritin isfound in many body cells, especially inthe Liver, Slpeen Bone Marrow and in Reticuloendothelial cells. Ferritin playsa singnificant role in the absorption storage and release of iron. Ferritinpredicts and monitors iron deficiency, differentiate iron deficiency fronchronic disease as cause of Anemia.

--End Of Report--

Patient Name		**Lab No**	1409222
Reg No/IP No	101301186 / 26/110098	**Sample Date**	11/02/2026 22:21
Age/Gender	38 Yrs/Female	**Receiving Date**	12/02/2026 06:21
Bed No/Ward	General Medicine	**Report Date**	12/02/2026 11:01
Referred By	Dr. General Medicine Unit III	**Report Status**	Final

Test Name	Result	Unit	Biological Ref. Range	Method
LIVER FUNCTION TEST (LFT)				
Sample: Serum				
Bilirubin Total (TBil)	**1.35 H**	mg/dl	0.20 - 1.20	Diazonium Salt
Bilirubin indirect	0.94	mg/dl	0 - 1	Direct Measure
Bilirubin Direct (Dbil)	0.41	mg/dl		CALCULATED
SGOT (AST)	**52 H**	IU/L	14 - 36	Enzymatic Colorimetric
SGPT (ALT)	26	IU/L	0 - 35	UV with P5P
Alkaline Phosphatase	46	IU/L	38 - 126	pnpp,amp Buffer
Protein Total	7.60	g/dl	6.30 - 8.20	Biuret Method
Albumin	4.40	g/dl	3.50 - 5.00	BCG Dye Method
Globulin	**3.20 H**	g/dl	2.90 - 3.10	Calculated
A/G Ratio (Albumin/Globulin Ratio)	1.38	g/dl	1.00 - 2.00	CALCULATED
GGT	<10	U/L	12 - 43	Gama-Glutamyl P Nitroanilide

--End Of Report--

Patient Name		**Lab No**	1409222
Reg No/IP No	101301186 / 26/110098	**Sample Date**	11/02/2026 22:21
Age/Gender	38 Yrs/Female	**Receiving Date**	12/02/2026 06:21
Bed No/Ward	General Medicine	**Report Date**	12/02/2026 11:01
Referred By	Dr. General Medicine Unit III	**Report Status**	Final

Test Name	Result	Unit	Biological Ref. Range	Method
CRP (QUANTITATIVE)				
Sample: Serum				
CRP (Quantitative)	4.3	mg/L	0.0 - 5.0	Serum, Immunoturbidimetry
IRON DEFICIENCY PROFILE				
Sample: Serum				
Iron	149.0	ug/dl	37.0 - 170.0	PYRIDYL AZO DYE
Total Iron Binding Capacity (TIBC)	**228.00 L**	mg/dl	265.00 - 497.00	PYRIDYL AZO DYE
Transferrin	**139.4 L**	mg/dl	176 - 280	CALCULATED
% Transferrin Saturation	**65.3 H**	%	20 - 50	CALCULATED

--End Of Report--

R
[redacted] 038Y, F, 10130186
Chest - PA, 12-02-2026, 02:39:00
[redacted]